Eastside Community Health Clinic AI use policy

Draft for board review · Effective when the board approves it · Owner: Not yet named; the compliance committee reviews it and the board approves it · Next review: Date for the board to set; the scribe rollout decision comes after the pilot, in about three months.

Why we have this policy. We want AI to take paperwork off staff, like charting at night. We must protect patient information and keep people making every decision.

Who it covers. Staff, clinicians, volunteers, contractors and board members at both clinics.

TOOLS WE USE

- **AI scribe from our health record vendor:** Visit notes for the three pilot providers; signed business associate agreement, so it may hold patient information.
- **OpenEvidence and UpToDate AI search:** Clinical questions only; no patient information.
- **ChatGPT:** General letters and handouts; no patient, donor or staff information, even names.
- **New tools, add-ons and AI features.** Approvers are listed below. Anything that could touch patient information needs a signed business associate agreement first, no exceptions.

WHAT YOU CAN USE AI FOR

- Look up a clinical question, like first-line treatment, with no patient information
- Draft patient handouts for clinician review, and reminder or sliding fee letters with no names
- Pilot providers: let the scribe draft visit notes, after the patient agrees

NEEDS APPROVAL FIRST

- Any new tool or AI feature that could touch patient information — *Nadia, privacy and security officer*
- Any other new tool, add-on or AI feature — *Beth, chief operating officer*
- Any clinical use of AI — *Dr. Okafor, medical director*

OFF LIMITS

- **Never put this into an AI tool** unless the tool is approved for it above: Diagnoses, medications, behavioral health notes, substance use treatment records, HIV status, reproductive health, immigration status, sliding fee income documents, insurance numbers, staff files, and the donor list (some donors are patients).
- Never put patient information into ChatGPT, Google Translate or any tool not approved for it above.
- Don't use AI in secret. If you're unsure, ask your manager.

CHECK EVERYTHING BEFORE IT GOES OUT

- Providers read every word of a scribe note before signing. AI has added medications we decided against.
- A clinician reviews AI-drafted handouts for facts, names and numbers. Whoever sends AI-assisted work is responsible for it.

DECISIONS ABOUT PEOPLE

- A clinician decides and signs all care, and a person decides sliding fee eligibility. AI may only look things up or draft for review.

BEING OPEN ABOUT AI

- Tell patients when the scribe is listening and let them say no, using the verbal consent script.
- If we add a chatbot, patients must be told it is not a person.

IF SOMETHING GOES WRONG

- Tell Nadia right away if patient information may have gone into an AI tool. She runs the HIPAA breach clock.
- Reporting a mistake is expected, not punished.

WHO OWNS THIS POLICY

- The compliance committee reviews this policy, then the board approves it. Your manager explains it; ask if unsure.

OPEN QUESTIONS FOR THE BOARD

- After the pilot, do we roll the scribe out to all providers, and should behavioral health use it at all?
- Do we formally approve ChatGPT, OpenEvidence and UpToDate AI search? Beth called the scribe our only approved AI.
- Who checks AI-assisted Spanish materials, may we use Google Translate, and may AI suggest billing codes? Who approves?
- How will we answer funders about AI, reassure staff about phone bots, and set the policy owner and review schedule?

Sample policy for an invented organization, drafted by Fulton Ring from a mock 30-minute call. Not legal advice; not reviewed by a real nonprofit.